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Department of Medicine

UHID	MRN30454	IP Number	IP-1637
Patient Name	[redacted]	Admitting Doctor	[redacted]
Age/gender	47 Y,9 M,18 D/Female	Admitting Doctor Speciality	Medicine
Mobile No.	[redacted]	Treating Doctor	[redacted]
Bed No./Ward	2/ICU	Treating Doctor Speciality	Medicine
Payer Name	Free	departMentNameForEMR	Medicine
Admission Date Time	12-03-2026 02:00 PM	Admission Type	Medical
Discharge Date Time	19-03-2026 11:55 AM	Discharge Type	Refer External Hospital
Status	Final		

Transfer Discharge summary

Diagnosis Type	ICD	Diagnosis	Priority Type	Note
FINAL	SEVERE ANEMIA SECONDARY TO BLEEDING HEMORRHOIDS WITH HIGH OUTPUT HEART FAILURE, CONGESTIVE HEPATOPATHY WITH HYPOTHYROIDISM	SEVERE ANEMIA SECONDARY TO BLEEDING HEMORRHOIDS WITH HIGH OUTPUT HEART FAILURE, CONGESTIVE HEPATOPATHY WITH HYPOTHYROIDISM		

Case Summary

A 47-year-old female patient presented to the emergency department with complaints of generalized weakness, giddiness since morning, exertional dyspnea on walking, and bleeding per rectum during defecation. She has a past history of hemorrhoidectomy done 2 years ago and gives a history of intermittent per rectal bleeding since then, for which she had previously received 5–6 units of blood transfusion. There is no history of diabetes mellitus, hypertension, or alcohol use; however, she has a history of chronic tobacco chewing for the past 15 years.

On examination, the patient was conscious, oriented, pallor +++, and hemodynamically borderline with blood pressure of 94/61 mmHg, pulse rate 93/min, respiratory rate 21/min, and SpO# 100% on room air. Systemic examination revealed normal heart sounds, bilateral clear air entry in lungs, soft and non-tender abdomen, and no focal neurological deficit.

Laboratory investigations revealed severe anemia with hemoglobin of 1.4 g/dL, RBC count of 0.55 million/#L, and thrombocytopenia with platelet count of 75,000/#L. Iron studies showed markedly reduced serum ferritin (1.50 ng/mL), suggestive of severe iron deficiency. Thyroid profile revealed elevated TSH (14.85) with low T3 (0.43). Peripheral smear was suggestive of dimorphic anemia with microcytic hypochromic picture along with macrocytes, anisopoikilocytosis, pencil cells, tear drop cells, and target cells. Renal parameters showed mildly elevated serum creatinine (1.9 mg/dL).

In view of severe symptomatic anemia with ongoing per rectal bleeding, the patient was admitted to ICU and transfused with 4 units of packed red blood cells (approximately 350 ml each). Parenteral iron therapy was administered as injectable iron sucrose according to calculated iron deficit (1518 mg).

Supportive management included proton pump inhibitors, antifibrinolytics, stool softeners, multivitamin supplements, hematinics, and antipyretics.

Surgical evaluation was done, and proctoscopy revealed internal hemorrhoids at 3, 5, and 9 o'clock positions, with one prolapsed hemorrhoid at 5 o'clock position; however, no active bleeding source was identified at the time of examination. Surgical management in the form of band ligation was advised after correction of anemia. Gynecological opinion was also taken to rule out per vaginal bleeding, and no active gynecological intervention was required; ultrasound abdomen and pelvis was advised.

Post-transfusion, hemoglobin improved to 5.10 g/dL; however, the patient continued to have bleeding per rectum during defecation. Further blood transfusion was advised, but due to non-availability of blood, transfusion could not be continued. The patient was counseled regarding blood donation and the need for further transfusion support. At present, the patient is not fit for surgical intervention due to persistent severe anemia.

In view of ongoing blood loss, severe anemia requiring further transfusion, and need for definitive surgical management after stabilization, the patient is being referred to GMC CHANDRAPUR for further evaluation and management including blood transfusion and subsequent definitive treatment of hemorrhoids. The patient's relatives have been explained in detail regarding the clinical condition, need for urgent management, and possible risks involved.

Investigations

Test Name	Result	Reference Range	Verification Date Time	Interpretation (H/L)
SERUM POTASSIUM	4.5	3.6-5.2	18 March 2026 17:13	
SERUM CREATININE	1.9	0.6-1.2	18 March 2026 17:14	H
COMPLETE BLOOD COUNT (CBC)			18 March 2026 17:13	
KIDNEY FUNCTION TEST (KFT)			18 March 2026 17:13	
SERUM SODIUM	129	125-154	18 March 2026 17:13	

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			18 March 2026 17:13	
LIVER FUNCTION TEST (LFT)		0.3-1.2	18 March 2026 17:13	
SERUM BILIRUBIN TOTAL	0.86	13-50	18 March 2026 17:13	H
BLOOD UREA	67	12-17	18 March 2026 17:13	L
Hemoglobin	5.10	0.6-1.2	18 March 2026 17:13	H
SERUM CREATININE	1.90	4-10	18 March 2026 17:13	H
WBC	10.93	<0.3	18 March 2026 17:13	H
SERUM BILIRUBIN - DIRECT (CONJUGATED)	0.63	0.0-0.7	18 March 2026 17:13	
SERUM BILIRUBIN - INDIRECT (UNCONJUGATED)	0.23	40-80	18 March 2026 17:13	H
Neutrophils	81.10	20-40	18 March 2026 17:13	L
Lymphocytes	10.60	<40	18 March 2026 17:13	H
SGOT, ASPARTATE AMINOTRANSFERASE (AST)	41	<45	18 March 2026 17:13	
SGPT, ALANINE AMINOTRANSFERASE (ALT)	17	2-10	18 March 2026 17:13	
Monocytes	7.00	1-6	18 March 2026 17:13	
Eosinophils	1.10	<128	18 March 2026 17:13	H
SERUM ALKALINE PHOSPHATASE	151	0-1	18 March 2026 17:13	
Basophils	0.20	6.4-8.3	18 March 2026 17:13	L
SERUM TOTAL PROTEIN	5.7	3.8-5.5	18 March 2026 17:13	L
RBC	1.81	3.5-5.5	18 March 2026 17:13	L
SERUM ALBUMIN	2.6	2.0-3.5	18 March 2026 17:13	
SERUM GLOBULIN	3.10	36-50	18 March 2026 17:13	L
PACKED CELL VOLUME OR HEMATOCRIT (PCV OR HCT)	15.60	1.1-2.5	18 March 2026 17:13	L
A/G RATIO	0.84	83-101	18 March 2026 17:13	
MCV	86.20	27-32	18 March 2026 17:13	
MCH	28.20	31.0-34.5	18 March 2026 17:13	
MCHC	32.70	11.6-14.0	18 March 2026 17:13	H
RDW-CV	29.90	40-55	18 March 2026 17:13	H
RDW-SD	80.60	150-410	18 March 2026 17:13	
PLATELET COUNT	150.00	6.5-12.0	18 March 2026 17:13	
Mean Platelet Volume (MPV)	9.20			

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Plateletcrit (PCT)	0.03	0.108-0.282	18 March 2026 17:13	L
PDW	19.20	10.0-17.9	18 March 2026 17:13	H
P-LCC	11.00	30-90	18 March 2026 17:13	L
P-LCR	38.60	11-45	18 March 2026 17:13	
THYROID FUNCTION TEST (TFT)			17 March 2026 16:37	
T3 (TRIIODOTHYRONINE) - TOTAL	0.43	0.55-1.8	17 March 2026 16:37	L
T4(THYROXINE) - TOTAL	5.93	4.66-10.5	17 March 2026 16:37	
THYROID STIMULATING HORMONE (TSH)	14.85	0.25-5.0	17 March 2026 16:37	H
COMPLETE BLOOD COUNT (CBC)			16 March 2026 12:32	
PERIPHERAL SMEAR EXAMINATION	**RBC Series**-Predominately Microcytic blood picture with moderate Hypochromia. Anisocytosis seen in the form of Macrocytes and normocytes. **WBC Series** - Adequate in number and morphology. **PLATLET Series** - Adequate in number and morphology.		16 March 2026 12:32	
Hemoglobin	6.20	12-17	16 March 2026 12:32	L
WBC	8.25	4-10	16 March 2026 12:32	
Neutrophils	79.60	40-80	16 March 2026 12:32	
Lymphocytes	13.40	20-40	16 March 2026 12:32	L
Monocytes	4.90	2-10	16 March 2026 12:32	
Eosinophils	1.60	1-6	16 March 2026 12:32	
Basophils	0.50	0-1	16 March 2026 12:32	
RBC	2.20	3.8-5.5	16 March 2026 12:32	L
PACKED CELL VOLUME OR HEMATOCRIT (PCV OR HCT)	20.50	36-50	16 March 2026 12:32	L
MCV	93.10	83-101	16 March 2026 12:32	
MCH	28.10	27-32	16 March 2026 12:32	
MCHC	30.20	31.0-34.5	16 March 2026 12:32	L
RDW-CV	25.00	11.6-14.0	16 March 2026 12:32	H
RDW-SD	73.10	40-55	16 March 2026 12:32	H
PLATELET COUNT	140.00	150-410	16 March 2026 12:32	L
Mean Platelet Volume (MPV)	10.50	6.5-12.0	16 March 2026 12:32	
Plateletcrit (PCT)	0.10	0.108-0.282	16 March 2026 12:32	L

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PDW	19.60	10.0-17.9	16 March 2026 12:32	H
P-LCC	40.00	30-90	16 March 2026 12:32	
P-LCR	42.20	11-45	16 March 2026 12:32	
SERUM CREATININE	1.45	0.6-1.2	15 March 2026 10:07	H
LIVER FUNCTION TEST (LFT)			15 March 2026 10:07	
COMPLETE BLOOD COUNT (CBC)			15 March 2026 10:07	
SERUM BILIRUBIN TOTAL	1.46	0.3-1.2	15 March 2026 10:07	H
Hemoglobin	5.50	12-17	15 March 2026 10:07	L
WBC	8.05	4-10	15 March 2026 10:07	
SERUM BILIRUBIN - DIRECT (CONJUGATED)	0.38	<0.3	15 March 2026 10:07	H
Neutrophils	83.30	40-80	15 March 2026 10:07	H
SERUM BILIRUBIN - INDIRECT (UNCONJUGATED)	1.08	0.0-0.7	15 March 2026 10:07	H
SGOT, ASPARTATE AMINOTRANSFERASE (AST)	20	<40	15 March 2026 10:07	
Lymphocytes	10.60	20-40	15 March 2026 10:07	L
SGPT, ALANINE AMINOTRANSFERASE (ALT)	14	<45	15 March 2026 10:07	
Monocytes	3.60	2-10	15 March 2026 10:07	
Eosinophils	1.90	1-6	15 March 2026 10:07	
SERUM ALKALINE PHOSPHATASE	168	<128	15 March 2026 10:07	H
SERUM TOTAL PROTEIN	6.5	6.4-8.3	15 March 2026 10:07	
Basophils	0.60	0-1	15 March 2026 10:07	
RBC	2.09	3.8-5.5	15 March 2026 10:07	L
SERUM ALBUMIN	3.1	3.5-5.5	15 March 2026 10:07	L
PACKED CELL VOLUME OR HEMATOCRIT (PCV OR HCT)	18.40	36-50	15 March 2026 10:07	L
SERUM GLOBULIN	3.40	2.0-3.5	15 March 2026 10:07	
MCV	87.80	83-101	15 March 2026 10:07	
A/G RATIO	0.91	1.1-2.5	15 March 2026 10:07	L
MCH	26.30	27-32	15 March 2026 10:07	L
MCHC	30.00	31.0-34.5	15 March 2026 10:07	L
RDW-CV	26.10	11.6-14.0	15 March 2026 10:07	H

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RDW-SD	73.90	40-55	15 March 2026 10:07	H
PLATELET COUNT	112.00	150-410	15 March 2026 10:07	L
Mean Platelet Volume (MPV)	9.70	6.5-12.0	15 March 2026 10:07	
Plateletcrit (PCT)	0.11	0.108-0.282	15 March 2026 10:07	
PDW	20.30	10.0-17.9	15 March 2026 10:07	H
P-LCC	40.00	30-90	15 March 2026 10:07	
P-LCR	35.60	11-45	15 March 2026 10:07	
COMPLETE BLOOD COUNT (CBC)			14 March 2026 07:00	
Hemoglobin	3.60	12-17	14 March 2026 07:00	L
WBC	5.76	4-10	14 March 2026 07:00	
Neutrophils	77.50	40-80	14 March 2026 07:00	
Lymphocytes	14.00	20-40	14 March 2026 07:00	L
Monocytes	5.70	2-10	14 March 2026 07:00	
Eosinophils	1.70	1-6	14 March 2026 07:00	
Basophils	1.10	0-1	14 March 2026 07:00	H
RBC	1.44	3.8-5.5	14 March 2026 07:00	L
PACKED CELL VOLUME OR HEMATOCRIT (PCV OR HCT)	12.00	36-50	14 March 2026 07:00	L
MCV	83.30	83-101	14 March 2026 07:00	
MCH	25.30	27-32	14 March 2026 07:00	L
MCHC	30.40	31.0-34.5	14 March 2026 07:00	L
RDW-CV	26.80	11.6-14.0	14 March 2026 07:00	H
RDW-SD	69.70	40-55	14 March 2026 07:00	H
PLATELET COUNT	108.00	150-410	14 March 2026 07:00	L
Mean Platelet Volume (MPV)	10.00	6.5-12.0	14 March 2026 07:00	
Plateletcrit (PCT)	0.11	0.108-0.282	14 March 2026 07:00	
PDW	20.90	10.0-17.9	14 March 2026 07:00	H
P-LCC	41.00	30-90	14 March 2026 07:00	
P-LCR	38.10	11-45	14 March 2026 07:00	
COMPLETE BLOOD COUNT (CBC)			13 March 2026 08:27	
Hemoglobin	2.60	12-17	13 March 2026 08:27	L
WBC	4.25	4-10	13 March 2026 08:27	

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Neutrophils	77.20	40-80	13 March 2026 08:27	
Lymphocytes	16.00	20-40	13 March 2026 08:27	L
Monocytes	5.10	2-10	13 March 2026 08:27	
Eosinophils	1.00	1-6	13 March 2026 08:27	
Basophils	0.70	0-1	13 March 2026 08:27	
RBC	1.07	3.8-5.5	13 March 2026 08:27	L
PACKED CELL VOLUME OR HEMATOCRIT (PCV OR HCT)	8.80	36-50	13 March 2026 08:27	L
MCV	82.50	83-101	13 March 2026 08:27	L
MCH	24.60	27-32	13 March 2026 08:27	L
MCHC	29.80	31.0-34.5	13 March 2026 08:27	L
RDW-CV	28.40	11.6-14.0	13 March 2026 08:27	H
RDW-SD	71.40	40-55	13 March 2026 08:27	H
PLATELET COUNT	107.00	150-410	13 March 2026 08:27	L
Mean Platelet Volume (MPV)	10.10	6.5-12.0	13 March 2026 08:27	
Plateletcrit (PCT)	0.11	0.108-0.282	13 March 2026 08:27	
PDW	18.70	10.0-17.9	13 March 2026 08:27	H
P-LCC	42.00	30-90	13 March 2026 08:27	
P-LCR	39.70	11-45	13 March 2026 08:27	
HHH TEST			12 March 2026 15:45	
SERUM ELECTROLYTES			12 March 2026 15:45	
NT PRO BNP	3253	<480	12 March 2026 15:45	
LACATED DEHYDROGENASE (LDH)	152		12 March 2026 15:45	
Sickling (Rapid test for Hb S & Hb A)	NEGATIVE		12 March 2026 15:45	
ABO BLOOD GROUPING	AB Positive		12 March 2026 13:05	
PERIPHERAL SMEAR EXAMINATION	**RBC Series** - Predominantly microcytic blood picture with severe hypochromia and few macrocytes and ovalocytes. Marked anisopoikilocytosis also seen in the form of pencil cells, Tear drop cells and target cells. **WBC Series** - Adequate in number and morphology. **PLATELETS Series** - Adequate in number and morphology.		12 March 2026 13:05	
ABO BLOOD GROUPING	[redacted]		12 March 2026 12:06	
LIVER FUNCTI (LFT)	[redacted]			

			12 March 2026 12:06	
URINE ROUTINE			12 March 2026 12:06	
LIPID PROFILE			12 March 2026 12:06	
SERUM FERRITIN	<1.50		12 March 2026 12:06	
RENAL FUNCTION TEST (RFT)			12 March 2026 12:06	
COMPLETE BLOOD COUNT (CBC)			12 March 2026 15:45	
HIV 1 AND 2 DETECTION TEST	Non Reactive	125-154	12 March 2026 15:45	
SERUM SODIUM	134		12 March 2026 12:06	
URINE-COLOR	PALE YELLOW	<200	12 March 2026 12:06	
SERUM CHOLESTEROL	59	0.3-1.2	12 March 2026 12:06	
SERUM BILIRUBIN TOTAL	0.49	13-50	12 March 2026 12:06	
BLOOD UREA	37	12-17	12 March 2026 12:06	L
Hemoglobin	1.40		12 March 2026 15:45	
HBSAG CARD TEST	Negative	3.6-5.2	12 March 2026 15:45	L
SERUM POTASSIUM	3.48	4-10	12 March 2026 12:06	
WBC	5.68		12 March 2026 12:06	
URINE APPEARANCE	Clear	0.6-1.2	12 March 2026 12:06	H
SERUM CREATININE	1.90	<0.3	12 March 2026 12:06	
SERUM BILIRUBIN - DIRECT (CONJUGATED)	0.18	<150	12 March 2026 12:06	
SERUM TRIGLYCERIDES	41		12 March 2026 15:45	
HCV ANTIBODY CARD TEST	Negative	97-111	12 March 2026 15:45	
SERUM CHLORIDE	105		12 March 2026 12:06	
Urine pH	5.5	30-85	12 March 2026 12:06	L
HDL CHOLESTEROL	18	3.5-7.2	12 March 2026 12:06	H
SERUM URIC ACID	12.0	0.0-0.7	12 March 2026 12:06	
SERUM BILIRUBIN - INDIRECT (UNCONJUGATED)	0.31	40-80	12 March 2026 12:06	
Neutrophils	69.00		12 March 2026 12:06	
Urine SG	1.010	<40	12 March 2026 12:06	
SGOT, ASPARTATE AMINOTRANSFERASE (AST)	22	<130	12 March 2026 12:06	
LDL CHOLESTEROL	34	20-40	12 March 2026 12:06	
Lymphocytes	25.60			

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URINE-GLUCOSE	NIL		12 March 2026 12:06	
SGPT, ALANINE AMINOTRANSFERASE (ALT)	10	<45	12 March 2026 12:06	
Monocytes	3.80	2-10	12 March 2026 12:06	
VLDL CHOLESTEROL	8.20	0-30	12 March 2026 12:06	
URINE-ALBUMIN	NIL		12 March 2026 12:06	
SERUM ALKALINE PHOSPHATASE	173	<128	12 March 2026 12:06	H
TOTAL CHOLESTEROL /HDL RATIO	3.28	1-5	12 March 2026 12:06	
Eosinophils	1.10	1-6	12 March 2026 12:06	
URINE FOR KETONE BODIES	Absent		12 March 2026 12:06	
SERUM TOTAL PROTEIN	5.5	6.4-8.3	12 March 2026 12:06	L
Basophils	0.50	0-1	12 March 2026 12:06	
SERUM LDL/HDL CHOLESTEROL RATIO	1.89	0.0-3.55	12 March 2026 12:06	
Pus Cells	2-3		12 March 2026 12:06	
URINE MICROSCOPY				
RBC	0.55	3.8-5.5	12 March 2026 12:06	L
SERUM ALBUMIN	2.9	3.5-5.5	12 March 2026 12:06	L
Epithelial cells	1-2		12 March 2026 12:06	
PACKED CELL VOLUME OR HEMATOCRIT (PCV OR HCT)	5.40	36-50	12 March 2026 12:06	L
SERUM GLOBULIN	2.60	2.0-3.5	12 March 2026 12:06	
URINE RBCs	NIL		12 March 2026 12:06	
MCV	97.00	83-101	12 March 2026 12:06	
A/G RATIO	1.12	1.1-2.5	12 March 2026 12:06	
MCH	25.30	27-32	12 March 2026 12:06	L
Bacteria	ABSENT		12 March 2026 12:06	
Remarks	-		12 March 2026 12:06	
MCHC	26.10	31.0-34.5	12 March 2026 12:06	L
RDW-CV	20.20	11.6-14.0	12 March 2026 12:06	H
RDW-SD	74.80	40-55	12 March 2026 12:06	H
PLATELET COUNT	175.00	150-410	12 March 2026 12:06	
Mean Platelet Volume (MPV)	9.90	6.5-12.0	12 March 2026 12:06	
Plateletcrit (PCT)	0.17	0.108-0.282	12 March 2026 12:06	

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		10.0-17.9	12 March 2026 12:06	H
PDW	18.50	30-90	12 March 2026 12:06	
P-LCC	64.00	11-45	12 March 2026 12:06	
P-LCR	36.40			

Advice
In view of ongoing blood loss, severe anemia requiring further transfusion, and need for definitive surgical management after stabilization, the patient is being referred to GOVERMENT HOSPITAL GADCHIROLI for further evaluation and management including blood transfusion and subsequent definitive treatment of hemorrhoids. The patient's relatives have been explained in detail regarding the clinical condition, need for urgent management, and possible risks involved.

Treatment on Discharge

Item Name	Frequency	Days	Route	Meal Relation	Composite	Instruction
NUROKIND OD TAB. 10'S(10 TAB)	OD	7			0dosage 0unit	
TAB ELTROXIN 100 MCG	OD	15		Before Breakfast	0dosage 0unit	
FERROUS ASCORVATE+FAOLIC A (XT) TAB. 10'S(10 TAB)	OD	15		After Dinner	0dosage 0unit	
FOLIC ACID TAB 30'S(30 TAB)	OD	15		After Breakfast	0dosage 0unit	
SYP CREMAFFIN	Bed Time	1		After Dinner	0dosage 0unit	15 ML HS
ANOBLISS OINTMENT	BD	1	Local Application		0dosage 0unit	
PANTOPRAZOLE 40 MG TAB 10'S(10 TAB)	OD	5		Before Breakfast	dosage unit	
TAB. PAUSE 500 MG	TDS	5			0dosage 0unit	
PARACETAMOL 500 TAB. (1 * 15)	SOS	3		After Meal	0dosage 0unit	

Print Date & Time
19-03-2026 12:02 PM